

1. Administrative & Study Identification

Brief Study Title: A Study to See How Well and How Safely Different Treatments Work in a Group of Participants With Non-Small Cell Lung Cancer (NSCLC) **Official Full Study Title:** A Phase I-III, Multicenter Study Evaluating the Efficacy and Safety of Multiple Therapies in Cohorts of Patients With Resectable Stage I-III Non-Small Cell Lung Cancer, Selected According to Biomarker Status **Short Title/Acronym:** HORIZON 2 **Posting ID:** 686714250083806900 **Protocol Number:** B043249 / 2024-511239-91-00 **NCT Number:** NCT06624059 **Sponsor Name:** Roche (Hoffmann-La Roche) **Trial Status:** Active but not currently recruiting (ACTIVE_NOT_RECRUITING) **Investigational Product:** Alectinib, Cisplatin (e.g., Platinol), Carboplatin, Pemetrexed **Phase of Development:** Phase 3 **Medical Monitor:** Hoffmann-La Roche Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy and/or safety of multiple therapies in patients with early-stage resectable NSCLC. Cohort B1 evaluates the safety and efficacy of alectinib in combination with up to four cycles of platinum-based chemotherapy in the adjuvant setting post complete surgical resection. Cohort B2 evaluates the efficacy and safety of perioperative alectinib in combination with chemotherapy in the neoadjuvant setting. **Secondary Objectives:** To assess investigator-assessed disease-free survival (DFS), overall survival (OS), and the incidence of adverse events (AEs).

2.2 Background & Rationale Non-small cell lung cancer (NSCLC) is the most common type of lung cancer, and while surgery is the primary treatment for resectable cases, recurrence remains a significant risk. This study addresses the need to improve outcomes by combining targeted therapy (alectinib for ALK fusion-positive NSCLC) with platinum-based chemotherapy before and/or after surgery. This approach aims to reduce tumor size pre-surgery and prevent cancer recurrence post-surgery.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Parallel Assignment (Non-Randomized) **Blinding:** None (Open-label) **Randomization:** Non-Randomized

3.2 Planned Enrollment & Duration Target \$\$\$: 11 participants
Number of Sites: Multicenter global study **Estimated Study Start:** October 2024 **Estimated Primary Completion:** Up to 8 years for long-term survival follow-up

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria Cohort B1: 1. Complete resection of the primary NSCLC with negative margins. 2. Confirmed stage II to select stage IIIB (T3N2) NSCLC of non-squamous (adenocarcinoma) histology. 3. Eastern Cooperative Oncology Group (ECOG) performance status of 0 or 1.

Cohort B2: 1. Evaluation by the operating attending surgeon and involved medical oncologist prior to study enrollment to verify study eligibility for complete surgical resection with curative intent. 2. Pathologically and/or histologically confirmed Stage II-IIIA and IIIB (T3N2 only) NSCLC of non-squamous (adenocarcinoma) histology.

Cohorts B1 and B2 (Both): 1. Documented ALK fusion.

4.2 Exclusion Criteria Cohort B1: 1. NSCLC of squamous or mixed histology regardless of the presence of an ALK mutation. 2. Prior exposure to any systemic anti-cancer therapy.

Cohort B2: 1. NSCLC of squamous or mixed histology regardless of the presence of an ALK mutation. 2. Known sensitivity to any component of alectinib, pemetrexed, cisplatin, or carboplatin. 3. Prior exposure to any systemic anti-cancer therapy.

Cohorts B1 and B2 (Both): 1. Pregnancy or breastfeeding, or intention of becoming pregnant during the study.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Cohort B1 (Adjuvant):** Intravenous (IV) platinum-based chemotherapy (Cisplatin/Carboplatin + Pemetrexed) on Day 1 of each 3-week cycle for up to 4 cycles, PLUS oral alectinib twice daily (BID) for up to 5 years. * **Cohort B2 (Neoadjuvant/Perioperative):** Oral alectinib BID and IV chemotherapy on Day 1 of each 3-week cycle for up to 3 cycles prior to surgery, followed by alectinib monotherapy for up to 5 years post-surgery.

Route of Administration: Oral (alectinib) and Intravenous (chemotherapy). **Duration of Treatment:** Up to 5 years of treatment.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care medications during active chemotherapy and post-surgical recovery. **Prohibited:** Prior systemic anti-cancer therapy

or treatment with other targeted therapies matching the biomarker profile.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	2 to 4 weeks prior to Tx	Informed Consent, Biomarker testing (ALK fusion), NSCLC staging
Neoadjuvant (Cohort B2)	9 weeks	Up to 3 cycles (3 weeks each) of alectinib + chemotherapy
Surgery	N/A	Complete surgical resection of the tumor
Adjuvant (Cohort B1/B2)	12 weeks	Up to 4 cycles (3 weeks each) of alectinib + chemotherapy
Monotherapy	Up to 5 years	Alectinib monotherapy, continuous efficacy and safety monitoring

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints * **Cohort B1:** Incidence, type, and severity of adverse events (AEs) with onset up to 28 days after the last dose of chemotherapy. * **Cohort B2:** Investigator-assessed pathologic complete response (inv-pCR).

7.2 Secondary & Exploratory Endpoints * **Cohort B1:** Investigator-assessed disease-free survival (DFS). * **Cohort B1:** Overall survival (OS). * **Cohort B1:** Incidence, type, and severity of AEs with onset up to 28 days after the last dose of study treatment.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via regular clinical visits during the infusion periods and oral therapy follow-ups. Focus on known chemotherapy side effects and targeted therapy tolerability. Monitored closely for up to 28 days following the last dose of treatment. **Serious Adverse Events (SAEs):** Evaluated per protocol and reported to regulatory bodies within standard reporting timelines (typically 24 hours). **Data Monitoring Committee (DMC):** Internal and external safety assessments managed by the sponsor (Roche).

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy outcomes and Safety Set for the incidence of adverse events. **Sample Size Justification:** Target enrollment of $N=11$ appropriately powered to evaluate safety and efficacy signals in parallel non-randomized cohorts. **Handling of Missing Data:** Standard oncological imputation techniques and right-censoring rules applied for survival data (DFS and OS).

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Onsite and remote monitoring to ensure protocol compliance, especially during the combination treatment phases and post-surgery evaluations. **Audit Trail:** Secured eCRF data entry, central biomarker verification, and regular clinical trial auditing by the sponsor.

11. Study Identifiers & Governance

Posting ID: 686714250083806900 **Primary ID:** B043249 **Registry Number:** NCT06624059 **Sponsor/Lead Organization:** Roche (Hoffmann-La Roche) **Study Brief Title:** A Study to See How Well and How Safely Different Treatments Work in a Group of Participants With Non-Small Cell Lung Cancer (NSCLC) **Official Regulatory Title:** A Phase I-III, Multicenter Study Evaluating the Efficacy and Safety of Multiple Therapies in Cohorts of Patients With Resectable Stage I-III Non-Small Cell Lung Cancer, Selected According to Biomarker Status

12. Clinical Framework (NSCLC Specific)

Primary Condition: Non-Small Cell Lung Cancer (NSCLC) (Metastatic / Resectable early-stage) **Diagnosis Threshold:** Stage II to IIIB (T3N2) with documented ALK fusion **Medical Methodology:** Perioperative or adjuvant combination of ALK-targeted therapy and platinum-based chemotherapy **Optimization Target:** Pathologic complete response (inv-pCR), prolonged disease-free survival (DFS), and overall survival (OS)

13. Study Procedures & Methodology

Management Pathway: Surgical resection combined with pre- or post-operative systemic therapies **Education Protocol (HCP):** Site initiation training on biomarker-driven patient selection and open-label drug administration protocols **Education Protocol (Patient):** Informed consent, understanding of the open-label nature of the trial, and awareness of targeted therapy plus chemotherapy side effects **Quality Audit Mechanism:** Source data verification, pathological margins review, and centralized biomarker status confirmation

14. Observation & Follow-Up Schedule

Total Duration: Up to approximately 10 years, including long-term follow-up **Onsite Visit Cadence:** Every 3 weeks during the active chemotherapy cycles, followed by regular intervals during the 5-year alectinib monotherapy phase **Remote Monitoring Frequency:** Regular safety phone calls and tele-health follow-ups post-treatment **Checklist Review Interval:** Standard oncology follow-up intervals for recurrence and survival checks

15. Sign-off & Approval

Role	Name	Signature	Date						
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]						
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]						
Statistician	[Name]	_____	[DD-MMM-YYYY]						